

City General Hospital

Department of Internal Medicine
550 Medical Center Drive, Berlin 10117 | Tel: +49 30 1234-5678

PATIENT MEDICAL REPORT

Patient Name:	Schmidt, Johann Karl	Date of Birth:	14 March 1968
Patient ID:	CGH-2024-008821	Gender:	Male
Admission Date:	02 May 2026	Discharge Date:	09 May 2026
Attending Physician:	Dr. Anika Brauer, MD	Ward:	Internal Medicine — Ward 3B

1. Chief Complaint

The patient presented to the emergency department with a 3-day history of progressive shortness of breath, productive cough with yellowish sputum, fever (38.9°C), and pleuritic chest pain localized to the right lower lobe region. The patient also reported fatigue and reduced appetite over the preceding week.

2. Medical History

Past Medical History:

The patient has a documented history of Type 2 Diabetes Mellitus (diagnosed 2014, currently managed with Metformin 1000 mg twice daily), arterial hypertension (managed with Ramipril 5 mg once daily and Amlodipine 5 mg once daily), and mild chronic obstructive pulmonary disease (COPD, GOLD Stage I) diagnosed in 2019. No prior hospitalizations for pulmonary conditions. No known drug allergies.

Family History:

Father deceased at age 72 due to myocardial infarction. Mother alive, age 78, with a history of Type 2 Diabetes and hypertension. One sibling with no significant medical history.

Social History:

Former smoker (20 pack-year history, quit 2018). Occasional alcohol use (1-2 units per week). Works as a retired civil engineer. Lives with spouse. Fully vaccinated including influenza (October 2025) and pneumococcal vaccine (2022).

3. Physical Examination on Admission

Vital Sign	Value	Reference Range	Status
Temperature	38.9 °C	36.1 – 37.2 °C	HIGH
Heart Rate	102 bpm	60 – 100 bpm	HIGH

Blood Pressure	138 / 88 mmHg	< 130 / 80 mmHg	ELEVATED
Respiratory Rate	24 breaths/min	12 – 20 breaths/min	HIGH
SpO2 (room air)	93 %	95 – 100 %	LOW
Blood Glucose	9.4 mmol/L	4.0 – 7.8 mmol/L	HIGH
Weight / BMI	84 kg / 27.3 kg/m2	18.5 – 24.9 kg/m2	OVERWEIGHT

Respiratory examination revealed dullness to percussion and reduced breath sounds over the right lower lobe with bronchial breathing and coarse crepitations. Cardiovascular, abdominal, and neurological examinations were unremarkable.

4. Investigations

Laboratory Results (02 May 2026):

Test	Result	Unit	Reference	Flag
WBC (White Blood Cells)	14.8	x10 ⁹ /L	4.0 – 11.0	H
Neutrophils	82	%	40 – 75 %	H
CRP (C-Reactive Protein)	128	mg/L	< 5 mg/L	H
Procalcitonin	1.8	ng/mL	< 0.5 ng/mL	H
Haemoglobin	13.2	g/dL	13.5 – 17.5 g/dL	L
Platelets	312	x10 ⁹ /L	150 – 400	N
Serum Creatinine	96	umol/L	62 – 115 umol/L	N
eGFR	72	mL/min/1.73m2	> 60	N
HbA1c	7.8	%	< 7.0 %	H
Sodium	136	mmol/L	135 – 145 mmol/L	N
Potassium	3.9	mmol/L	3.5 – 5.1 mmol/L	N
Lactate Dehydrogenase	298	U/L	135 – 225 U/L	H
Blood Urea Nitrogen	9.2	mmol/L	2.5 – 7.1 mmol/L	H

Imaging:

Chest X-Ray (PA, 02 May 2026): Right lower lobe consolidation consistent with pneumonia. No pleural effusion. Cardiac silhouette within normal limits. No pneumothorax.

CT Thorax with contrast (03 May 2026): Confluent consolidation in the right lower lobe with air bronchograms. No cavitation. No pulmonary embolism. Mild mediastinal lymphadenopathy. Incidental note of mild emphysematous changes bilaterally consistent with known COPD.

Microbiology:

Sputum culture (02 May 2026): Streptococcus pneumoniae isolated, sensitive to Amoxicillin, Ceftriaxone, and Levofloxacin. Blood cultures (x2 sets): No growth after 5 days. Urinary Legionella antigen: Negative. Urinary Pneumococcal antigen: Positive.

5. Diagnosis

#	Diagnosis	ICD-10 Code	Status
1	Community-Acquired Pneumonia (CAP), right lower lobe — Streptococcus pneumoniae	J18.1	Primary
2	Type 2 Diabetes Mellitus — suboptimal glycaemic control	E11.9	Active
3	Arterial Hypertension — controlled	I10	Active
4	COPD, GOLD Stage I — stable	J44.1	Active

6. Treatment Plan

Antibiotic Therapy:

Initiated IV Ceftriaxone 2g once daily on 02 May 2026. Following sputum culture sensitivity results, de-escalated to oral Amoxicillin-Clavulanate 875/125 mg twice daily on 05 May 2026. Total planned antibiotic course: 7 days (completion date: 09 May 2026).

Supportive Therapy:

Supplemental oxygen via nasal cannula (2–4 L/min) titrated to maintain SpO2 > 95%. Nebulised Salbutamol 2.5 mg every 6 hours PRN for bronchospasm. IV fluid resuscitation (Normal Saline 0.9%, 500 mL over 4 hours on admission). Deep vein thrombosis prophylaxis with Enoxaparin 40 mg subcutaneous once daily.

Diabetes Management:

Continued Metformin 1000 mg twice daily. Sliding scale insulin initiated for in-patient glucose management targeting fasting glucose 6–10 mmol/L. Endocrinology review requested for HbA1c optimisation prior to discharge. Consider addition of SGLT-2 inhibitor at outpatient follow-up.

7. Clinical Progress

The patient showed gradual clinical improvement over the admission period. Fever resolved by day 3 (04 May 2026). Oxygen requirements decreased progressively; supplemental oxygen was discontinued on 06 May 2026 with sustained SpO2 of 96–98% on room air. Cough and sputum production reduced significantly by day 5. Repeat CRP on 07 May 2026: 28 mg/L (down from 128 mg/L). Repeat chest X-ray on 08 May 2026 demonstrated partial resolution of right lower lobe consolidation. The patient was ambulatory and tolerating oral intake well by day 6.

8. Discharge Plan

Discharge Medications:

Medication	Dose	Route	Frequency	Duration
Amoxicillin-Clavulanate	875/125 mg	Oral	Twice daily	Until 09 May 2026
Metformin	1000 mg	Oral	Twice daily	Ongoing
Ramipril	5 mg	Oral	Once daily (morning)	Ongoing
Amlodipine	5 mg	Oral	Once daily (morning)	Ongoing
Salbutamol inhaler	100 mcg	Inhaled	PRN (max 4x/day)	Ongoing

Follow-Up Instructions:

1. General Practitioner review in 1 week (by 16 May 2026) for clinical reassessment and repeat inflammatory markers. 2. Repeat chest X-ray in 6 weeks to confirm full resolution of consolidation and exclude underlying malignancy. 3. Endocrinology outpatient appointment within 4 weeks for HbA1c management review and consideration of SGLT-2 inhibitor therapy. 4. Pulmonology referral for COPD reassessment and spirometry review within 3 months. 5. Patient educated on pneumonia prevention, smoking cessation resources offered (patient already a non-smoker), and importance of annual influenza vaccination reinforced.

Attending Physician:	Dr. Anika Brauer, MD	Report Date:	09 May 2026
Specialty:	Internal Medicine	Digital Signature:	Dr. A. Brauer — CGH-2026-0509
License No.:	DE-MED-2009-04421	Report Status:	FINAL — VERIFIED